Supplement Table 3: Findings of studies examining the association between occupational sitting and cardiovascular mortality[1]

Nr.	Study	Sample	Major occupational groups	Follow-up length/ duration	Assessment of occupational sitting (as analysed)	Outcomes	Findings[2]	Study quality[3]	Sex-stratified analysis
	First author, year, country, cohort	n (% female) age at baseline	General population or specification		Categories of measurement and standardized instrument (if applicable)	(Data basis)	Statistical results and reference categories (e.g. risk estimates with 95 % confidence interval)	(++) (+) (0)	Yes/no
1.	Chau 2015 Norway HUNT3	n=36817 (45 % F); (for occupational sitting) 18–>60 y	General population (incl. working)	(2006–2008) mean 3.3 y	Self-reported OPA in 4 categories: 'work that mostly involves sitting'; 'work that requires much walking', 'work that requires much walking and lifting'; 'heavy physical labour'	Cardiovas-cular mortality (death register)	Reference: mostly sitting much walking: HR=0.62 (0.31–01.24) walking and lifting: HR=0.77 (0.37–1.59) heavy physical labour: HR=0.55 (0.83–2.90) **No difference in risk of cardiovascular mortality for participants with mostly sitting jobs compared to jobs with more OPA.**	++	No
2.	Hayashi 2016 Japan Japan Collabo-rative Cohort Study for Evaluation for Cancer Risk (JACC Study)	n=66161; (54.6 % F); 40–79 y mean age: 55.6 y	General population (incl. working)	19.2 y median	Self-reported OPA in 4 categories: mostly sitting, sitting/standing, mostly standing, standing/walking	cardiovascular mortality (death certificates/ national register)	Reference: mostly sitting M standing/walking: HR=1.05 (0.94–1.17) standing/walking: HR=0.91 (0.81–1.04) M mostly standing: HR=1.20 (1.00–1.45) F mostly standing:HR=1.24 (1.01–1.53) **Cardiovascular mortality risks among the sitting/standing and standing/walking groups did not differ from those of the mostly sitting groups, while the mostly standing groups showed an increased risk.** BMI >25 kg/m^2: HR=0.79 (0.66–0.94) BMI <25 kg/m^2: HR=1.06 (0.96–1.16) **In overweight individuals, standing/walking OPA was associated with a reduced risk compared to mostly sitting OPA.**	+	Yes
3.	Kim 2013 USA Multiethnic Cohort Study (MEC)	n=134596 (54.4 % F); 45–75 y	General population (incl. working)	13.7 y median	Self-reported work sitting time h/d analysed in 3 categories: <1 h/d; 1–4 h/d; >5 h/d	cardiovascular mortality (death certificates/ national register)	Reference: work sitting <1 h/d M long work sitting >5 h/d: HR=0.93 (0.82–1.05) F long work sitting >5 h/d: HR=1.12 (0.95–1.31) **Long work sitting was not associated with risk of cardiovascular mortality.**	+	Yes
4.	Moe 2013 Norway HUNT2	n=37300 (49.2 % F); mean 44.1 y	General population (incl. working)	(1995–2008) 12.4 y median	Self-reported OPA, analysed in 3 categories: 'mostly sedentary', 'much walking/ lifting', 'heavy physical work'	Cardiovas-cular mortality (death register)	Reference: much walking/lifting OPA without metabolic syndrome (MS) Mostly sedentary HR=1.19 (0.84–1.70) Mostly sedentary with MS HR=2.74 (1.82–4.12) **Compared to much walking/lifting OPA no clear associations between OPA and cardiovascular mortality for people without MS, but different associations for persons with MS**	+	No
5.	Stamatakis 2013 Great Britain Health Survey for England (HSE) Scottish Health Survey (SHS)	n=10834 (48.1 % F); >40 y	General population (incl. working)	Mean 12.9 y (+/- 3.3) y	Self-reported predominant OPA categorized into predominant sitting activity at work versus predominant standing/walking activity at work	Cardiovas-cular mortality (national register)	Reference: mainly sitting OPA Overall sample standing/walking OPA: HR=1.06 (0.75–1.49) F standing/walking OPA: HR=1.74 (0.86–3.51) M standing/walking OPA: HR=0.99 (0.69–1.41) **Occupational activity was not associated with cardiovascular mortality for men or women.**	+	Yes

Legend:
n (% F): number of participants (percent female)
Female=F; Male=M
OPA: occupational physical activity
1 in alphabetical order and after methodological assessment (high to low quality)
2 in model adjusted for age, sex, leisure time physical activity (LPTA), health behaviour (alcohol, smoking, diet), and socioeconomic status (education, household income) if available, unless reported otherwise
3 Methodological Assessment incl. risk of bias: assessment after the SIGN-checklists resulting in overall ratings of 'high quality' (++), 'acceptable quality' (+) and 'unacceptable/low quality' (0), synonymic with 'high quality' (++) / low risk of bias, 'acceptable quality' (+) / accepable risk of bias, and 'unacceptable/low quality' (0) / unacceptable risk of bias.